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TRIBUTE IN MOTION

WHITE PAPER
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The Unattended Hour

India registers ten million deaths a year and has no published standard for what happens in the hours after any of them. This is the argument for building one — and for building it around mobility.

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FOR STAKEHOLDERS, INVESTORS, INSTITUTIONS, PARTNERS, MEDIA,
AND THE HEALTHCARE AND ELDER-CARE ECOSYSTEMS

A NOTE ON EVIDENCE

Every figure in this paper is attributed to a named source in **Appendix A**. Where we rely on our own primary research we say so and describe exactly what it was. Figures we could not source to a public authority have been **excluded rather than softened** — including one we had previously used ourselves. They are listed, with the reason, on page 19. We would rather be checked than believed.

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The argument in six lines

The event is enormous and unmeasured. India registered 10,224,506 deaths in 2021. Only 23.4 per cent were medically certified. Three-quarters of registered deaths pass through no clinical record at all — which means the hours after them are, by default, unattended by any system.

The demand curve is already locked in. Indians aged 60 and over will exceed a fifth of the population by 2050, and will outnumber the country's children by 2046. The 80-plus population grows by roughly 279 per cent between 2022 and 2050. None of this is a forecast of behaviour; it is arithmetic on people already born.

The decision-maker is increasingly not in the room. India's diaspora is the world's largest at 35.42 million. Add internal migration and the nuclear household, and the person who must decide is often in another city, frequently in another time zone.

The failure is structural, not moral. The sequence after a death is a chain of handoffs between unrelated parties. Each break loses information the family must then re-supply — and one of those breaks is also the moment of sale.

Mobility is the only spine available. Everything else in the sequence can be re-ordered or deferred. The movement of the body cannot. Whoever holds the movement is the only party in a position to guarantee anything at all.

Regulators who examined this reached the same point. The UK's Competition and Markets Authority recommended an inspection and registration body specifically for *the transportation and care of the deceased*. Scotland now has a statutory register. India has no equivalent framework, and no one has published a standard to be measured against. That is the position Eternal Repos intends to occupy.

I

The unattended hour

Every consequential day in an Indian family's life has an industry standing behind it. A wedding has venues, planners, caterers, photographers, insurers and a price list. A birth has a hospital, a protocol, a discharge process and a bill you can read. The day someone dies, a family is handed a telephone.

What happens next is not organised by anyone. It is improvised — competently and kindly in many cases, chaotically in many others — by whoever happens to answer that phone. A relative who knows a man with a vehicle. A hospital attendant with a number. A neighbour who has done this before. The outcome depends almost entirely on who that person is and what kind of day they are having.

It is worth being precise about the gap, because it is easy to name it wrongly. The gap is *not* an absence of providers. India has a great many people who move the deceased, arrange rites, supply materials and staff crematoria. Many are skilled. Some are extraordinarily kind. The gap is the absence of a **standard**: nothing published, nothing measured, nothing a family can read beforehand, and therefore nothing anyone can be held to afterwards.

An industry without a published standard is not necessarily a bad industry. It is an unaccountable one. And unaccountability is most costly precisely where the customer is least able to shop, compare, complain or walk away — which is an accurate description of a family in the first hours after a death.

> *An hour a family will remember for thirty years is presently organised by whoever happens to answer the phone.*

II

What the record actually shows

India's Civil Registration System recorded **10,224,506 registered deaths in 2021**. Of those, 2,395,128 — **23.4 per cent** — carried a medical certification of cause of death. Even counting every case where the deceased received any medical attention at all during their terminal illness, the figure reaches only 49.5 per cent.

FIGURE 1 — THE CERTIFICATION GAP

23.4%

of India's registered deaths in 2021 were medically certified. The remaining 7.8 million passed through no clinical record of cause.



Medically certified · **2,395,128** Not certified · **7,829,378**

Why this matters more than it first appears. Certification is a proxy for institutional contact. A death that is not medically certified is overwhelmingly a death that happened at home, in a village, in a community — outside any building with a protocol. The first hours after three-quarters of India's registered deaths are therefore not merely badly served. They are structurally unattended: no institution is present whose job it is to do anything.

Source: Report on Medical Certification of Cause of Death 2021, Office of the Registrar General of India.

This single ratio reframes the category. It is tempting to imagine the problem as a hospital problem — a mortuary, a discharge desk, a corridor. For a quarter of registered deaths, it is. For the rest, there is no corridor. There is a house, a family, and a set of decisions that begin immediately and cannot be postponed.

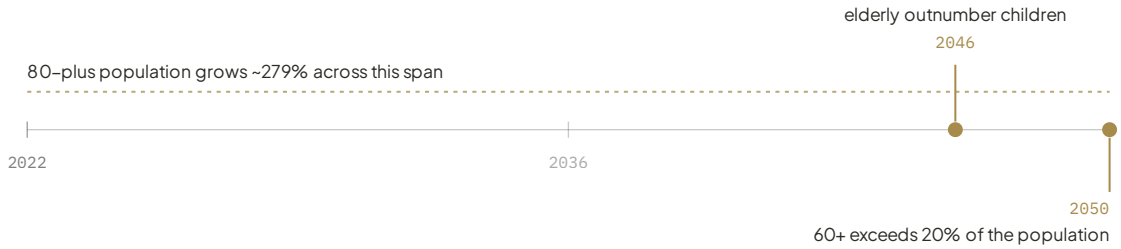
India's crude death rate stands at 7.5 per 1,000 — 10.1 in Chhattisgarh at the high end, 5.4 in Delhi at the low. The absolute number will rise for decades regardless of that rate, for the reason set out next.

III

The demographic turn

India is in the early part of a demographic transition it has not yet organised for. The relevant figures are not projections of behaviour; they concern people who are already alive and already counted.

FIGURE 2 — THREE MILESTONES ALREADY IN THE ARITHMETIC



The generation now in its forties will manage more farewells than any before it — from greater distances, with fewer hands, and with parents who live longer and die older.

Source: India Ageing Report 2023, UNFPA India.

Two further figures from the same report shape what the service has to be able to do. Roughly **71 per cent of older persons live in rural areas**, which puts the majority of future demand outside the metros where private services concentrate. And **18.7 per cent of India's elderly have no income of their own**, which should permanently discipline any temptation to treat this as a luxury category.

Ageing is also feminising. Since 1991 the elderly sex ratio has risen consistently: women live longer, and are correspondingly more likely to be widowed, and more likely to face the arrangements of a death with fewer economic resources than the person who died. The typical decision-maker in this category is not who the marketing of this sector assumes.

IV

Distance

The second structural change is geographic. India's diaspora is the largest in the world: **35.42 million people** — 15.85 million non-resident Indians and 19.57 million people of Indian origin, as counted by the Ministry of External Affairs. Layer internal migration on top of that, and the nuclear urban household on top of that.

The operational consequence is specific and under-appreciated. In a growing share of Indian deaths, **the person who must make the decisions is not in the room** — and often not in the country. They are being told by telephone that a parent has died in a city they left years ago. They must authorise arrangements they cannot see, for rites they may not have performed themselves, through a provider they cannot assess, while booking a flight.

This is why the category is a mobility category rather than a facility category. The distance between the deceased and the decision-maker has to be closed by someone, physically, within hours. That closing *is* the service.

Everything a family in that position actually wants — *go there, be competent, tell me what you find, do it in our tradition, and do not make me negotiate from an airport* — is a description of a logistics standard, not a product catalogue.

> *The distance between the deceased and the decision-maker has to be closed by someone, physically, within hours.*

It also explains why repatriation and inter-city transfer are not edge cases in this business. They are the leading indicator of where the whole category is heading.

V

What a family is asked to carry

Grief is not the problem this business solves. Grief is not a problem, and any company that offers to reduce it should be distrusted. What can be taken off a family is everything standing *around* the grief — and in the current arrangement, that load is heavy, sequential, and arrives at the worst possible cognitive moment.

Two loads sit on the family at once.

THE OPERATIONAL LOAD

The repetition. A father's name said eleven times in one afternoon to eleven different strangers, because nobody wrote it down the first time. An address given, and given again. A relationship explained to a clerk, then to a driver, then to an attendant, then to a priest. None of these people are being unkind; none of them has any mechanism for receiving what the last one was told.

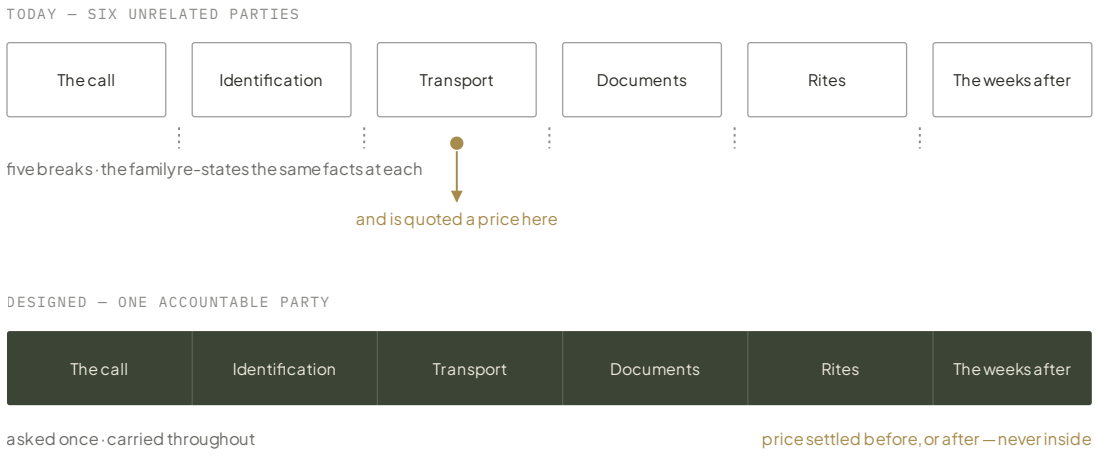
THE EMOTIONAL LOAD

Every one of those exchanges demands a decision, and they are being demanded in the worst decision-making state a human being will ever occupy. Bereavement measurably degrades judgement, recall and the capacity to weigh options. The sector's commercial arrangements currently depend on that state persisting for the duration of the sale.

That is the uncomfortable sentence in this paper, and we think it needs saying plainly rather than implied. The mechanism deserves to be drawn rather than described, because the failure is architectural rather than moral — which is also why it is fixable.

> *No party in the chain can promise anything about a stage it does not control.*

FIGURE 3 — WHERE ACCOUNTABILITY BREAKS



The same six stages occur in both rows. What changes is whether anyone is accountable across them. Today each junction is a break: the family re-supplies its own facts, and no party can promise anything about a stage it does not control. One of those junctions is also the moment of sale — a price is put to a family at the point of maximum distress and minimum ability to refuse. Moving that moment out of the sequence is not a courtesy. It is the structural fix, and it is only available to a party that holds the whole chain.

VI

Why mobility is the spine

Of everything in that sequence, exactly one element is non-negotiable, non-substitutable and time-bound: **the body has to move.**

Documentation can be re-ordered. Notification can be delegated. Rites can be scheduled to the family's tradition and the family's pace. Even the ceremony itself can be deferred by days while relatives fly in. The movement cannot be deferred, cannot be done remotely, and cannot be done badly without everyone noticing.

This has a consequence that is easy to miss and central to our strategy. **Whoever controls the movement controls the timeline.** Only a party physically present, in possession of the vehicle and the trained crew, is in a position to make a promise about when anything happens — and therefore only that party can offer a guarantee that means anything. An advisory service, an app, a marketplace or a call centre can route a family to a provider. None of them can commit to a response time, because none of them owns the thing that responds.

> *A service standard for the final journey has to be built on the vehicle, or it is a recommendation.*

This is not a proprietary insight. It is the same conclusion a competition regulator reached from an entirely different direction, examining an entirely different market, set out in Section X.

Ceremonial mobility is the name we give to the resulting category: purpose-designed vehicles, crews trained to a written standard of conduct, a published response time, and a record that travels with the family rather than being reconstructed at each stop. It is a logistics business with the conduct requirements of a hospice and the reliability requirements of an ambulance service.

VII

What is priced, and what is not

In 2026 we called ten funeral service operators across Delhi NCR and asked each what they sell and for how much. We were quoted packages between ₹14,500 and ₹30,000. This was ten telephone calls, not a market study, and we describe it that way deliberately.

What the exercise revealed was less about price than about what price is attached to. Every line item quoted to us is an object or a ritual input. Not one operator published a guaranteed response time. Not one published a hygiene protocol.

FIGURE 4 — THE TWO COLUMNS OF THIS MARKET

WHAT THE MARKET PRICES	WHAT THE FAMILY ACTUALLY EXPERIENCES
The vehicle, per trip	Whether anyone answered at 3 a.m.
The freezer box, per day	How long it took, against a number stated in advance
Cloth, wood, ghee, garlands	Whether the person who arrived knew their mother's name
The priest, per ceremony	Whether the handling was hygienic, to a written protocol
Transport to the ghat	Whether they were asked the same question twice
Storage, refrigeration, ambulance hire	Whether anything was sold to them in that hour
—	Whether anyone was there in the weeks that followed

The left column is a bill of materials. The right column is the service. Nothing in the right column appeared on any quotation we received.

Source: Eternal Repos, ten operator calls across Delhi NCR, 2026. Primary research; ten calls, not a survey.

A market that prices only objects will compete only on the price of objects. It has no mechanism to reward conduct, because conduct is not on the quotation and therefore cannot be compared, chosen or paid for. Publishing conduct — in measurable terms, before the sale — is the act that creates that mechanism.

VIII

Dignity, written as a specification

"Dignity" is the most-used and least-useful word in this sector. As ordinarily deployed it is a sentiment: unfalsifiable, unmeasurable, and therefore free. Every operator claims it and no operator can fail it.

Our position is that dignity only becomes an operating standard when it is written as a specification — a set of claims that are **stated in advance, measured afterwards, and capable of being failed.** A promise you cannot fail is not a standard; it is advertising.

Each of the seven commitments overleaf is testable. A response time either was or was not met. A protocol either is or is not published. A family either was or was not asked the same question twice. There is no interpretive latitude in any of them, which is the point.

We publish them knowing that publishing them is what makes it possible to hold us to them — and we publish them now, before we are large enough to be safe from them. An operator who commits to a standard only once it can comfortably meet it has not made a commitment. It has made an announcement.

> *A promise you cannot fail is not a standard. It is advertising.*

The limits are published in the same breath, and for the same reason. A service that will not say what it cannot do has not told you what it can.

PUBLISHED IN ADVANCE

Seven commitments

01 **A person, at any hour**

Never a menu. Never "call back in the morning."

02 **A published response time**

Stated before it is needed, and measured after.

03 **A written hygiene protocol**

Published in full, so anyone can read it before they trust us.

04 **Nothing asked twice**

What a family tells us once is recorded and carried.

05 **Nothing sold in that hour**

No options put to a family least able to refuse them.

06 **Conduct as a trained discipline**

Not a temperament we hope for, but a standard we teach and assess.

07 **Responsibility for the weeks that follow**

The service does not end when the vehicle leaves.

And the limits, stated plainly. We cannot lessen the meaning of goodbye. We cannot protect anyone from the truth of absence. We are not here to make grief beautiful, and we are not here to place luxury around loss. We are here for the ordinary floor of decency — a smaller claim, and a much harder one to keep.

IX

What hospitality lends, and where it stops

Eternal Repos borrows its operating system from hospitality, and this needs explaining carefully, because the obvious reading of that sentence is the wrong one.

What hospitality has genuinely solved, and this sector has not, is the industrialisation of conduct. A good hotel does not hope its staff will behave well. It defines the behaviour, trains it, measures it, and recovers from failures with a documented protocol.

THE FOUR THINGS WORTH TAKING

The twenty-four hour desk. A discipline of always answering, staffed and rostered as an operational commitment rather than a good intention.

The guest record. Information given once is held and travels with the guest, so nobody is asked their own name twice. This is precisely the mechanism missing from Figure 3.

The handover note. A structured transfer of context between shifts, so continuity survives the people.

The recovery protocol. A defined response for when something goes wrong, rather than improvisation by whoever is present.

AND THE THREE THINGS THAT MUST NOT COME WITH IT

The upsell. Ambience sold as a product. The vocabulary of luxury.

A hotel's commercial model rests on discretionary spending by people in a good mood with the freedom to decline. Every one of those conditions is inverted here. Importing hospitality's sales model into a bereavement service is not merely tasteless; it is a category error, and it is the specific failure mode that has made "premium" funeral services distrusted in every market where they have appeared.

> *Take the operating system. Refuse the sales model.*

X

Where regulation is going

The strongest external validation of this thesis comes from a jurisdiction with no connection to India and no interest in our company.

In England, Wales and Northern Ireland there is **no bespoke statutory regime for funeral directors**. Standards are set by trade bodies whose codes carry no legal force and whose membership is optional — a description that will be familiar to anyone who has looked at the Indian sector.

The United Kingdom's Competition and Markets Authority examined that market and, in 2020, recommended the establishment of **an inspection and registration body to monitor standards of the transportation and care of the deceased**, together with far greater price transparency.

We would draw attention to the exact words. A competition regulator, working from consumer-harm evidence rather than from any theory about grief, identified the point of failure as *transportation and care* — the movement of the body and the conduct around it. That is the same conclusion Section VI reaches from first principles, arrived at independently.

Scotland went further and legislated. Under the Burial and Cremation (Scotland) Act 2016, inspection regulations and a statutory code of practice came into force on **1 March 2025**, and a **Funeral Sector Register launched on 1 April 2025**, with a licensing scheme still in development.

India today has no comparable framework: no licence, no register, and no published service standard for this work. We do not present that as a scandal — it is simply where the country currently is, and it is where the United Kingdom was until very recently. But the direction of travel among jurisdictions that have examined the question is consistent, and it points towards inspection, registration and published standards.

The strategic reading is straightforward. An operator that has already published a standard, already measures against it, and already keeps the records to prove it is positioned favourably when that framework arrives — and is building the evidence base that shapes it in the meantime.

XI

Ten years out

We are building towards a specific and unglamorous end state, in three phases.

DEPTH BEFORE SPREAD

Get Delhi NCR genuinely right — the response time held and measured, the crews trained and assessed, the record kept — before a second city is discussed. This is not a category in which one learns on people.

ADOPTION BY INSTITUTIONS

Hospitals, hospices, palliative units, senior-living operators and insurers all encounter this event repeatedly and almost none have a protocol for it. The protocol is the product long before the vehicle is. An institution that adopts a published standard exports it to every family it serves.

THE STANDARD BECOMES THE EXPECTATION

The end state is that an Indian family assumes certain things without asking: that someone will answer; that the person who arrives will already know their mother's name; that the rites will be theirs, at their pace, in their tradition; and that nobody will be sold anything in that room.

> *If in ten years that standard is being met by people who have never heard of us, that is not a competitive failure. That is the argument, won.*

We are aware of how that reads in a document addressed partly to investors, so we will put the commercial case in the same plain terms. A category with ten million annual events, a demand curve fixed by demography, no incumbent brand, no published standard, no regulatory floor, and a regulator elsewhere already naming the failure point, is not a philanthropic proposition. It is an unusually well-defined one. The philosophy and the business case are the same sentence: **whoever makes the promise first, and can be measured on it, owns the category the day it becomes accountable.**

What we are asking of you

Different readers can do different things with this document. These are the specific asks.

INVESTORS	Test the thesis on its weakest joint, which is unit economics on a mobility-led model in a price-sensitive market — not on whether the need exists. We will share the operating data behind the seven commitments under NDA.
HOSPITALS & HOSPICES	Read our hygiene protocol and response standard, and tell us whether they would survive your procurement and your clinical governance. If they would not, we would rather know now.
SENIOR LIVING & ELDER CARE	You face this repeatedly and improvise each time. Let us look at what your staff currently do when a resident dies, and write the protocol with you. There is no charge and no obligation attached to that.
INSURERS	A death claim and a funeral begin with the same telephone call. Your claims function is measured on the one thing we publish. That is a natural integration and we would like to explore it.
PARTNERS	Adopt the published standard, or challenge it in public. Either strengthens it. We have no interest in being the only operator that meets it.
MEDIA	The story is not a company. It is that the most consequential hour in an Indian family's life is the only one with no standard attached to it — and that the figures in Section II have not been read this way before.

Appendix A

Evidence and method

FIGURE	CLAIM	SOURCE
10,224,506	Registered deaths in India, 2021	Report on Medical Certification of Cause of Death 2021, Office of the Registrar General of India
23.4%	Share of registered deaths medically certified; 49.5% counting any medical attention during terminal illness	MCCD 2021, ORGI
7.5	Crude death rate per 1,000; Chhattisgarh 10.1 highest, Delhi 5.4 lowest	Sample Registration System Statistical Report 2021, ORGI
20%+	Share of population aged 60 and over by 2050	India Ageing Report 2023, UNFPA India
279%	Growth in the 80-plus population, 2022 to 2050	India Ageing Report 2023, UNFPA India
2046	Year the elderly population is projected to overtake the child population	India Ageing Report 2023, UNFPA India
71%	Share of older persons living in rural areas	India Ageing Report 2023, UNFPA India
18.7%	Share of India's elderly with no income of their own	India Ageing Report 2023, UNFPA India
35.42m	Overseas Indians worldwide — 15.85m NRIs, 19.57m PIOs	Ministry of External Affairs, Government of India, May 2024
2020	CMA recommendation of an inspection and registration body for the transportation and care of the deceased	Funerals Market Investigation, Competition and Markets Authority (UK); House of Commons Library briefing CBP-10475
2025	Scottish inspection regulations and code of practice in force 1 March; Funeral Sector Register launched 1 April	Burial and Cremation (Scotland) Act 2016; HoC Library briefing CBP-10475
10 calls	Delhi NCR operator quotations ₹14,500–₹30,000; none published a response time or hygiene protocol	Eternal Repos primary research, 2026

Appendix A · continued

What we do not claim

The following are figures or claims we have deliberately left out, with the reason. We publish this list because a paper that shows only its strongest evidence has told you less than it appears to.

That 90 per cent of India's death-care sector is unorganised.

This figure circulates widely and we have used it ourselves in earlier material. We can find no public source that stands behind it, so we have removed it. The defensible version of the same point is that India has no licence, no register and no published standard for this work — a statement about institutions rather than an estimated share, and one that can be checked.

Any national market-size number.

The figures in circulation trace back to operator commentary without published method. If asked how large this market is, the honest answer is that nobody knows — and that this is itself part of the problem.

That our operator research is a study.

It was ten telephone calls in one metropolitan region in 2026. It is indicative and it is ours; it is not a survey and we do not present it as one.

That the certification gap measures where people die.

Medical certification is a proxy for institutional contact, not a direct measure of place of death, and registration coverage varies by state. We draw a structural inference from it, which we have tried to state as an inference.

Any outcome for grief.

We make no claim that this service reduces grief, shortens it, or improves bereavement outcomes. We are not aware of evidence that would support such a claim, and we would not make it if we were.

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We are here for the ordinary floor of decency — a smaller claim, and a much harder one to keep.

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